“Launch Meeting: Introductions and Purpose” (Brian McCloskey, Rick Stanton, Brad Power) [#1]

March 23, 2022
Brad Power

“My hope is to help Brian, myself, and patients to come to decide what the next best therapy is after the standard of care as we push into immunotherapies and combinations of conventional, targeted, and immunotherapies.” – Rick Stanton

“My objective is to determine what is going to be the next best therapy for me.“ – Brian McCloskey

Meeting Summary

Advanced prostate cancer patients Brian McCloskey and Rick Stanton introduced themselves, their medical history, and their objectives for the Prostate Cancer Lab.

- Brian McCloskey: 56-year-old tech marketing executive; diagnosed 5.5 years ago; on abiraterone; sees the challenge of personalizing his treatment as hinging on data and analytics; wants to bring cutting edge diagnostic data and integrate it to understand his disease and guide his treatment.
- Rick Stanton: bioinformatician with many years in the pharmaceutical industry, including at Human Longevity, Inc., and Amgen; Stage IV; on chemo; wants to bring emerging research diagnostic technologies to his treatment decisions.

CancerHacker Lab founder Brad Power moderated and shared an overview of the Cancer Patient Lab and its purpose.

- Building on the hackathon for advanced prostate cancer patient Bryce Olson in 2021.
- Lab process: attract patients, gather data, analyze data and find treatment options, define treatment strategy.
- Have attracted diverse companies and individuals willing to help: diagnosticians, bioinformaticians, treatment matching, patient advocates, and clinicians.
- Focus on accelerating the use of cutting edge diagnostics (RNA, proteomics, single cell, spatial analysis), which are currently “Research Use Only”, for clinical decision-making.

Several of the participants introduced themselves, including:

- Nik Schork: of TGen, an organization bridging between health data and clinical guidance - has developed a protocol, approved by an IRB, to grant patients access to research data about them.
- Rick Davis: leader of AnCan, a community of cancer patients - clarified treatment options.
- Lea Ann Biafora: oncology nurse, founder of Beacon Associates; Jason Sager: pediatric oncologist; Stacy Hurt: patient advocate, colorectal cancer survivor; Jeff Waldron: healthcare community connector; Jan Sobieralski: advanced prostate cancer patient.

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“Launch Meeting: Introductions and Purpose” (Brian McCloskey, Rick Stanton, Brad Power) [#1]

Meeting Notes

Brian McCloskey: I’m a 56-year-old father of three and husband. I was diagnosed five-and-a-half years ago. I have been on an interesting journey since. Professionally, I spent the better part of 25 years building personalized consumer experiences in e-commerce and high tech, and most recently managed marketing for the largest healthcare staffing firm in the country, AMN Healthcare. My background is very data and digitally oriented. When I was diagnosed with cancer I realized there were some opportunities in how we used data to better personalize the experience, and hoped to use my background in approaching my cancer. The essence is trying to understand my cancer before we move onto treating it. It’s the same as when we used lots of data to personalize consumer experiences. I haven’t seen anything that tells me that that analogy doesn’t hold in medicine. There’s a long way to go in medicine, and that is what this is about. How do we use data to more intelligently treat us?
My passion is surfing. I live in San Diego. I passed it onto my kids.

Rick Stanton: I’m a surfer as well, though not as good as Brian. I have two kids and a wonderful wife. My daughter moved to Hawaii to surf.

I’m Stage IV. I’m on chemo. I have gone through multiple rounds.

My prostate cancer was discovered after noting a jump in my PSA at a yearly checkup. I had no symptoms.

It turns out I have a CDK12 mutation, which is aggressive, but opens up some therapy options. My goal is like Brian’s. There are about 1,000 clinical trials going on for advanced prostate cancer. They are targeting roughly 25 or 30 molecular targets of the immune system or in the cancer cell. I and Brian are at or nearing the end of the standard of care for NCCN guidelines. The standard of care includes androgen (hormone) blocking drugs, and then you go on chemo. I am on my fifth round of chemo and need to know what to do when it fails. You can’t stay on chemo forever. I’m having tingling in my fingers.

My hope is to help Brian, myself, and patients to come to decide what the next best therapy is after the standard of care as we push into immunotherapies and combinations of conventional, targeted, and immunotherapies.

I have 3 or 4 doctors, so I am getting multiple counsel.

Different doctors have different clinical trial options.

I’m finding that none of these treatment options are defined by my state.

RNA seq is typically not used.

Mutations are used, e.g., my CDK12 mutation is being used to guide my therapy.

“Launch Meeting: Introductions and Purpose” (Brian McCloskey, Rick Stanton, Brad Power) [#1]

Tumor mutational burden is used and some basics.

I’m hoping to get guidance on which trial or therapy to try next, to either manage my disease well or find a cure.

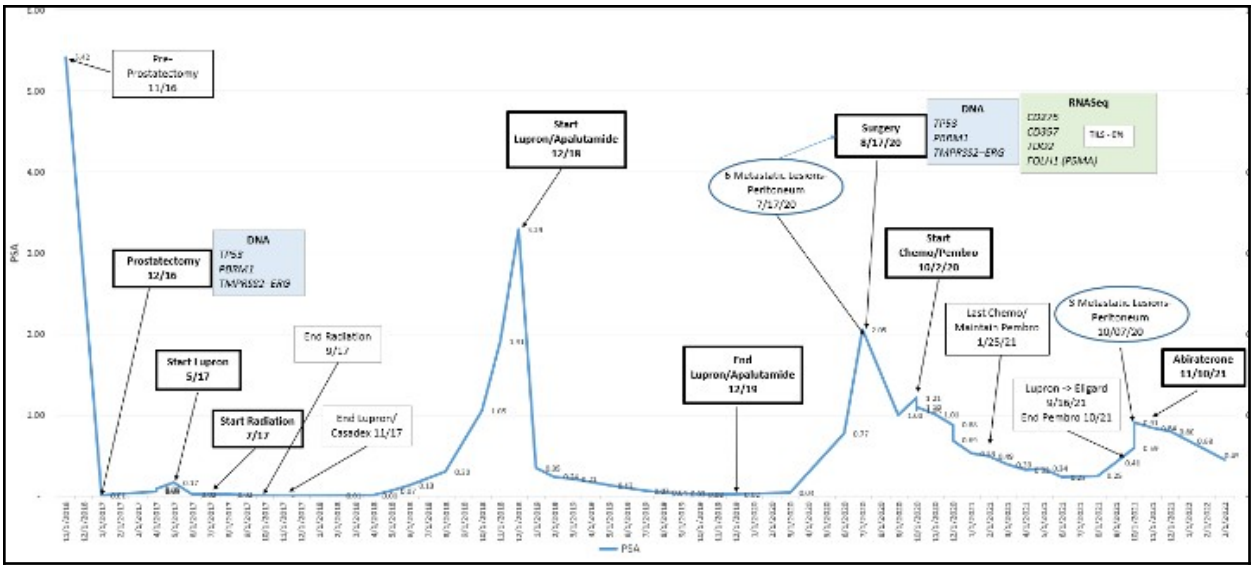
Brad Power: Any more details on your medical history? How has your PSA responded to different treatments?

Rick Stanton: I was diagnosed in late 2019. My treatments have been salvage radiation, Lupron, and Casodex, which controlled my PSA for 13 months. Then I went on darolutamide, which is a next-generation androgen receptor blocker. That didn’t do so much. I was on that for 4 months, and my PSA started rising to 3.6. My doubling time was about a month. It was not being controlled. It was determined I should go on docetaxel chemo, which I have been on since December. It brought me from 3.6 to 2.6, so only a marginal drop in my PSA. I have a nodal disease. I have 5 or 6 lymph nodes that have cancer in them, from my neck to my pelvis, as shown in a combination of CT and PSMA scans. I’m currently at a PSA of 2.6. My largest lymph node is about a centimeter, in the middle of my chest. I requested IHC stains from my primary tumor, which might guide my TILs and predict responsiveness to my tumor.

Brad: You might have noticed implicitly that Rick is a bioinformatician by background.

Brian McCloskey: Here is a graphic that describes my journey of treatments and responses:

Exhibit 1: Biomarker/Treatment Overlay - 8 Key Decisions



I was diagnosed in 2016. I had a radical prostatectomy. My DNA mutations are in the blue box: TP53, PBRM1, and TMPRSS2-ERG. I went on a number of different treatments: hormone therapy, radiation, a second line hormone therapy with apalutamide (decision number 4). Then I went on a holiday, and as soon as I went on a holiday, my disease came roaring back. My Gleason is a 4.3, with a tertiary grade 5, which would largely explain the aggressiveness of the

disease. I have 6 metastatic lesions in my peretenium. That was discovered in July of 2020. I had robotic surgery in August of 2020. We determined that my DNA mutations were the same as from my primary tumor in 2016.

Rick did an amazing job of quarterbacking an RNA seq analysis with Tempus.
There were a lot of findings from that; of note: expressions of CD276, CD357, TD02, and FOLH1 (PSMA). TILS 0%.

Definitely some challenges in terms of a very immunosuppressive environment.

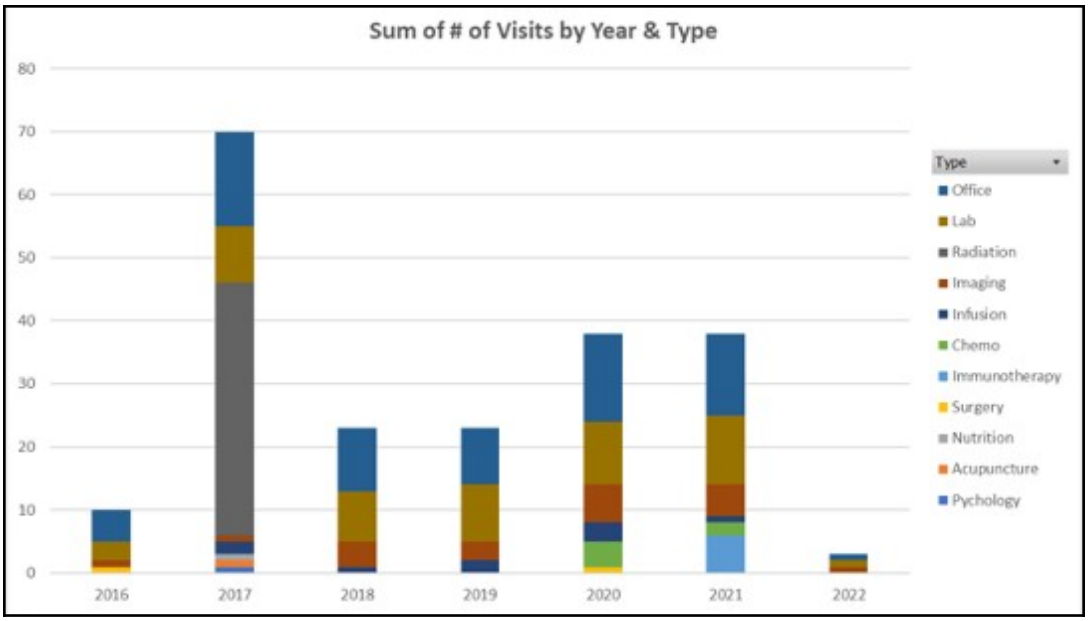
After my surgery, since we knew we didn’t get it all, I started on systemic therapy with chemo and pembrolizumab (Keytruda) in October 2020. 6 rounds. Then I remained on pembro until October 2021. Then my PSA began to rise and lesions re-appeared. I had “no evidence of disease” shortly after my surgery. I started abiraterone in November of 2021. My PSA was at .91, and it has dropped to a .45.

I am currently responsive to abi, which is great. But I don’t know how much mileage I’m going to get on this. I know that the likelihood of becoming hormone-insensitive is high.

My objective is to determine what is going to be the next best therapy for me.

Here is a quick snapshot of my journey in terms of the sheer number of medical visits:

Exhibit 2: 205 Cancer Visits over 5.5 years



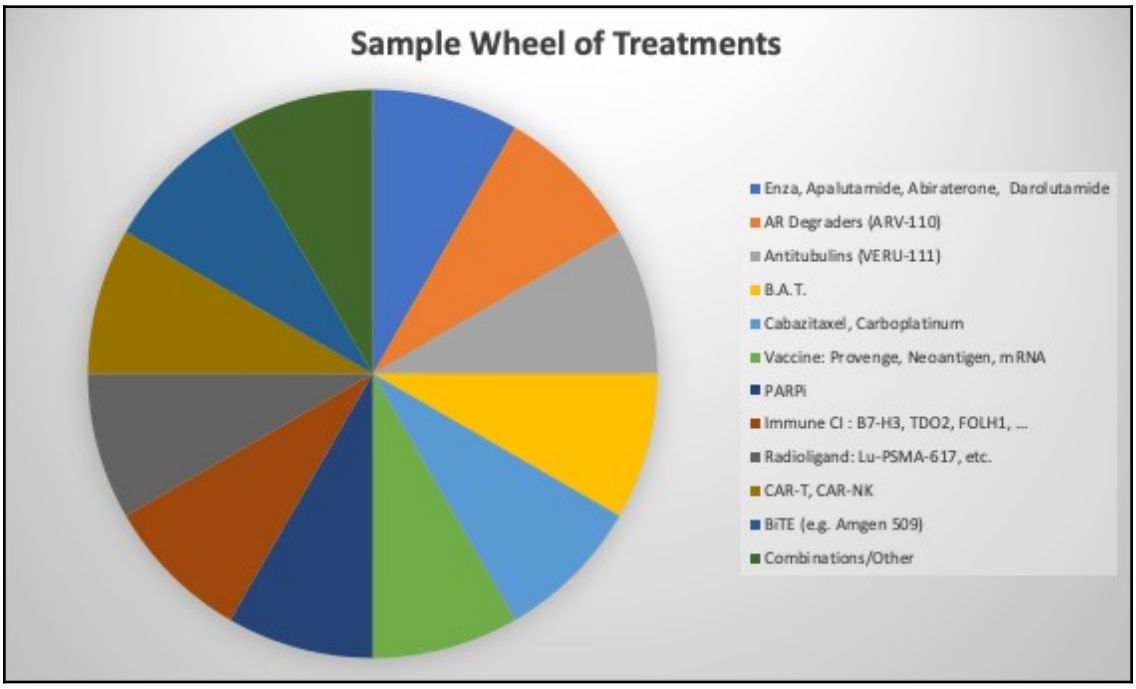
Office visits (dominant) - 33%
Lab - 25%
Radiation (a big part early on) - 20%

“Launch Meeting: Introductions and Purpose” (Brian McCloskey, Rick Stanton, Brad Power) [#1]

Imaging – 10%

Here is my table of treatment options:

Exhibit 3 - Treatment options



I want to get much smarter about which treatments I can rule out, and which treatments are going to be most relevant for me.

Today the way that decisions are made is largely based on the stage of the disease (e.g., metastatic), hormone-sensitive or not hormone-sensitive, and the medical history and relate that to clinical trials and mass treatment response rates.

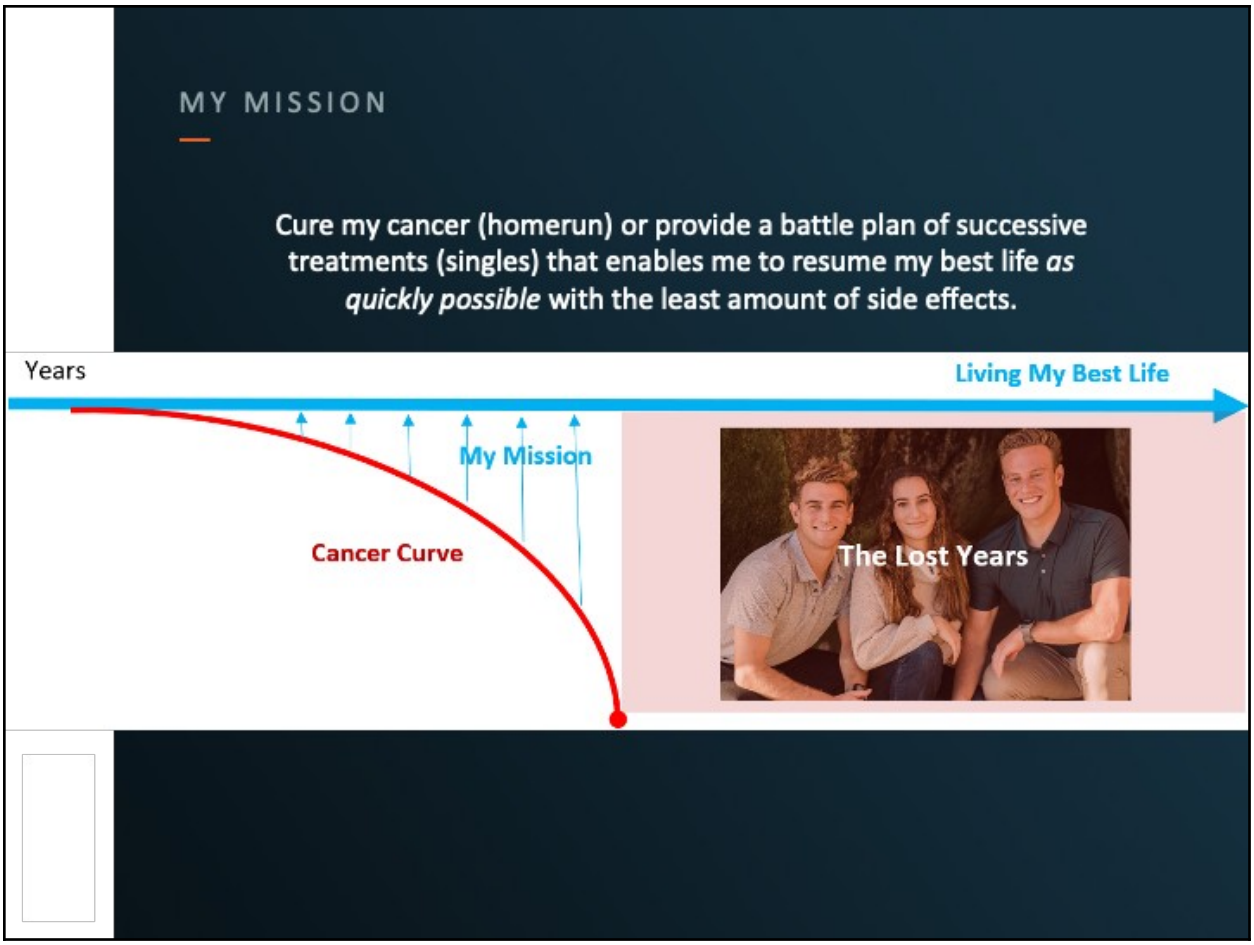
I believe we can do better than that.

We found some interesting insights from my DNA analysis. We had some interesting results from the RNA seq analysis. I wonder what else we can find if we continue to look from a genomic perspective, from a cell perspective, an extracellular perspective; look at my immune system, and maybe other factors, and help me do a better job to rule out treatments and prioritize those that will provide the greatest response.

My mission is to live as long as I can.

Exhibit 4 - My mission is to live my best life, as long as I can.

“Launch Meeting: Introductions and Purpose” (Brian McCloskey, Rick Stanton, Brad Power) [#1]



I am grateful for the amazing care I’ve had, and the advancements and the support I have received.

I also see there is a chance to accelerate the pace of translational medicine.

I’m thankful to have Rick and Brad on this journey.

I believe we are trying to better use the insights that exist in the lab and bring them to treat our diseases more effectively.

I have conversations every day with so many amazing doctors, scientists, and researchers about how those improvements are happening. I want us to take a different approach. This prostate cancer lab represents an opportunity to take a unique approach to do that.

Jan Sobieralski: I want to compliment Brian on his flowchart. This could help in any diagnosis. It’s a great way to present your case.

“Launch Meeting: Introductions and Purpose” (Brian McCloskey, Rick Stanton, Brad Power) [#1]

Brad Power: Prostate cancer is lucky in a way – it has a PSA as a measure of the aggressiveness of the disease which allows monitoring. Patients can effectively run experiments on themselves. For example, they try a drug, and it knocks the PSA down.

Stacy Hurt: Do either of you have any glaring comorbidities that would exclude you right off the bat from any clinical trials?

Brian McCloskey: Unfortunately, I’m a pretty healthy guy. I exercise all the time. It’s one thing I can control. I know Rick does too. You would look at me and not know I had cancer.

Rick Stanton: Same with me. I try to work out a lot. Swimming every day and lifting weights.

Rick Davis: Do we have any genitourinary oncologists on the group?

Rick Stanton: My primary care medical oncologist is Dr. Sandy Liu out of UCLA. Even though she couldn’t make it, she is very supportive, and she will be tracking our progress asynchronously as much as she can. Tanya Dorff at City of Hope, John Chen at UCLA, and Rana McKay at UCSD are my medical oncologists. I think they are awesome, they guide clinical trials, and are knowledgeable about options.

Brad Power: We have gotten commitments from six or eight clinicians who have committed to work with us but will not be on these calls every week.

Rick Davis: What are they each saying about where you are in your treatment, and what are they recommending that you do?

Rick Stanton: Dr. Liu is my primary medical oncologist because I am on an Arcus-6 clinical trial. Unfortunately I was assigned to the docetaxel chemo arm only. The clinical trial is run out of Arcus Biosciences. I know the CEO. I used to work for him when I was at Amgen, where I worked for 17 years in discovery research. I know the Arcus team very well. I used to consult to them. I saw the adenosine PD1 and docetaxel clinical trial as something that I wanted to explore. The trial is reaching its end.

I contacted Dr. Dorff for a second opinion. I’m trying to be proactive. She recommended a Xencor trial, which is a bispecific PD1 and CTLA4, along with olaparib.

I am also trying to get IHC stains for CD3, CD4, CD8, and PDL1. I’m waiting for that result.

Rick Davis: Do you have any markers for a PD1, or for olaparib? Is there any indication that those drugs might be suitable for you?

Rick Stanton: Only population statistics at this point.

“Launch Meeting: Introductions and Purpose” (Brian McCloskey, Rick Stanton, Brad Power) [#1]

Rick Davis: I’m asking because I get very concerned that guys who come to us are not used as lab rats. You’re getting a little bit of neuropathy, but that’s not unusual after five sessions. What about reducing the dose? Sticking where you are? Looking at what shows up right now on an NGS? What about adding a platinum if something unusual shows up? And in the meantime, science is moving so quickly. A year from now there could be something clear. Olaparib will do horrible things to your blood counts. Docetaxel is keeping you in check for now.

Rick Stanton: All of my doctors recommend that I stay on docetaxel for as long as possible. They are in complete agreement with you. They are very conservative.

By the way, I think you’re amazing. I have attended a number of AnCan meetings, and I see what you’re doing for the patients. Many are less informed.

Brian McCloskey: Rick [Davis], you may recall you were actually part of my last decision. When my PSA began to go up, we looked at abiraterone, enzalutamide, apalutamide - where I had an amazing response, and derolutamide.

I’m being treated at UCSD, and Rana McKay is my primary onc. She is incredible. I’m so fortunate to have her. I have many other medical oncologists that I speak to on a regular basis. We decided to go down the abiraterone route. The primary reason was that there were some clinical trials with a lot of success. We considered abi plus chemo. I had just come out of chemo, so that wasn’t the greatest fit. This is where it is part art and part science.

Where I want to focus this effort is not necessarily jumping to treatment decisions.

I’m fortunate that I am responding to abiraterone right now.

I have discussed all of the treatments on that list that I shared with Rana and my doctors.
For example, I just spent some time yesterday with Tanya Dorff, and we talked about when it would make sense to do a CAR-T.

I had an amazing conversation with Saul Priceman on my way home from City of Hope, and we talked about some really great opportunities to do some cutting edge diagnostics.
Right now, where I am in this honeymoon period, I don’t know how long it is going to last, I want to get a better handle on my disease. I want the data to speak. I’ve seen it in my professional career so many times. It’s like the cardinal sin of marketing: if you think you know who your customer is, you are wrong. You need to let them speak. You need to let the data speak.
Even though the work that Rick did on my RNA seq analysis wasn’t immediately processed by my oncology team, after three months, I got a thank you for it. “This is information that we can actually use for targeting.” Namely B7H3 and CD276.

I have a pretty good handle on what my DNA looks like, and a decent idea of what’s going on from an RNA perspective. That’s opened up new doors.

“Launch Meeting: Introductions and Purpose” (Brian McCloskey, Rick Stanton, Brad Power) [#1]

As we look at the flow of diagnostics, getting into proteomics and other things, what else am I going to uncover within those specific fields? And when you look across them, from a genomic perspective, a cell perspective, an extracellular perspective, immune, etc., how is that going to come together and provide new context for how my prostate cancer and Rick’s prostate cancer has evolved?

Hopefully we can identify more meaningful treatment options.

I have spoken to many, many people, researchers, oncologists, etc., and I’m very fortunate. For example, Laura Kleiman, who is on this call. I reach out to people and they pick up the phone. It’s crazy.

I am more and more convinced that what we are doing here is novel.

There is something kind of similar, “PROMISE” [Prostate Cancer Precision Medicine Multi-Institutional Collaborative Effort]. It’s a team of medical oncologists and researchers that are trying to correlate DNA to treatments. But I think that that is dealing with a limited set of information - DNA. How much of that is out there for prostate cancer patients? There is some. Yes: continue on that effort, but we really need to continue to look holistically at a whole range of diagnostic tools that can really shed light on our cancers.

Brad Power: Rick [Stanton] has been particularly useful in providing introductions into the new diagnostic area of spatial analysis, which is looking at the tumor microenvironment. We’ve had conversations with Akoya Biosciences, NanoString, and Enable Medicine, and these are all “Research Use Only” at this time. These are amazing tests that are being used to help with understanding the heterogeneity of the cancer and of the microenvironment. We want to accelerate its use for clinical guidance, for Rick, for Brian, and for other patients that would follow. And that will be a key part of this prostate cancer lab that we are launching, to see which of those tests are useful in clinical guidance. Part of our challenge is overcoming the hurdle of tests which are currently for “Research Use Only” and being able to use them for clinical guidance. Rick has been negotiating with these companies to get clinical access.

Rick Stanton: I’d like to introduce a wonderful organization, TGen, along these lines. Danielle, Sarah, Brian, Byron, and Nik Schork from TGen are on the call today. They have been supporting me in a most unusual way due to my friendship with Nik.

Nik Schork: Brad, good to reconnect. This is very inspirational.

As Brad pointed out, and as I think everyone on the call knows, one limitation of putting data on the desk of a physician to use is if, in fact, the data has a “research use only” designation, there are liabilities for what the physician can do. That is a constraint we’re all operating under. However, as Rick is pointing out, we put together a protocol, which was approved by an ethical body, to provide “research use only” and other data types back to Rick, so he could do with that data as he pleases, including putting it in front of a hackathon group like this.

“Launch Meeting: Introductions and Purpose” (Brian McCloskey, Rick Stanton, Brad Power) [#1]

And something remarkable happened the other day, and this just shows Rick’s strength.
There were concerns about our protocol, what parameters we needed to operate under that the ethical body was considering.

I suggested that Rick speak to the Western IRB people, and he did that.

He was able to field their questions. They told us that we could provide data. We didn’t have reach-through, so we couldn’t police what was done with that data after it left TGen’s offices and showed up at Rick’s door. What we learned was remarkable.

Western IRB is a huge organization. They see many protocols.

Rick was the first patient to ever appear before this ethical body and give his two cents worth.
I see this as enormous progress. It doesn’t mean we have carte blanche to do whatever we want, but it is an opening and a recognition that if data is collected on a patient, then the patient should have the right to use that data in whatever form he sees fit.

One analogy we made is that there is legislation about the “right to try” for particular drugs.
If you’re off guidelines, and you have a debilitating disease, there is legislation that would allow patients access to drugs that would otherwise not be available to them.

I said, if there is legislation that allows people to try drugs, that might be complicated, why isn’t there legislation to allow people to use data, which is even less complicated to use.
There has been substantial progress made by people like Rick who are willing to put themselves out there and appear before ethical bodies like Western IRB.

Brad, we are happy to trade notes in the future about how we are able to put this protocol together. We will share the template.

Sarah and Danielle are on the phone, if it could be recreated at other institutions.
It takes a little bit of time, but it gives us more freedom to operate.

Brad Power: Thanks, Nik. Nik is a hero for all of us in bridging the gap between data and clinical guidance for people. It is an honor to have you here.

I hope that what Nik, Brian, and Rick have been saying shows the potential of this group.
It has only become clear to me in the last few weeks that this patient-led initiative cuts through the typical regulatory protections that are well-meaning and in place, but they mean that these new diagnostic tests will eventually get into clinical use, but it might be five or eight year in the future. And we have patients in front of us right now who need to make a choice between – as Brian so eloquently laid out – a number of treatment options, and these tests might help guide what is most likely to provide benefit.

“Launch Meeting: Introductions and Purpose” (Brian McCloskey, Rick Stanton, Brad Power) [#1]

The patient, and especially the advanced cancer patient, who is facing a life and death decision, doesn’t have time to wait for the regulatory niceties.

That is what we are on top of.

If you look at the stakeholders around this decision, the diagnostic companies want to get their “research use only” technology into clinical use as fast as possible. The clinicians, Brian’s and Rick’s doctors, are quite happy to have information that will help them in their decisions, as Brian pointed out when he brought RNA seq data to his doctor, which helped identify new treatments. They are onboard. We are cutting through and catalyzing an environment that most of the parties involved would like to see: help patients in their decisions.

Brian McCloskey: To add a little bit of color to that: so many of the conversations that a patient has in the clinic is about treatments. What are the different options? The conversation that I have never had, until last week, and I’m just beginning to get into is: what are we doing with my tissue to help inform my treatment?

We know that tissue is a scarce resource in the diagnostics landscape. How we use that tissue for diagnostics is critical. We not only need to have a treatment strategy, but we need a diagnostics strategy, which is rooted in this scarce resource of tissue. Yes, there are liquid biopsies, etc. Potentially organoids. I am on a lot of different paths right now, and some amazing technology that is happening. But the point is still that where I am trying to go is to shift the conversation to focus on understanding, and to understand you have got to have diagnostics. And to have diagnostics, you have to figure out what we’re going to do with tissue.

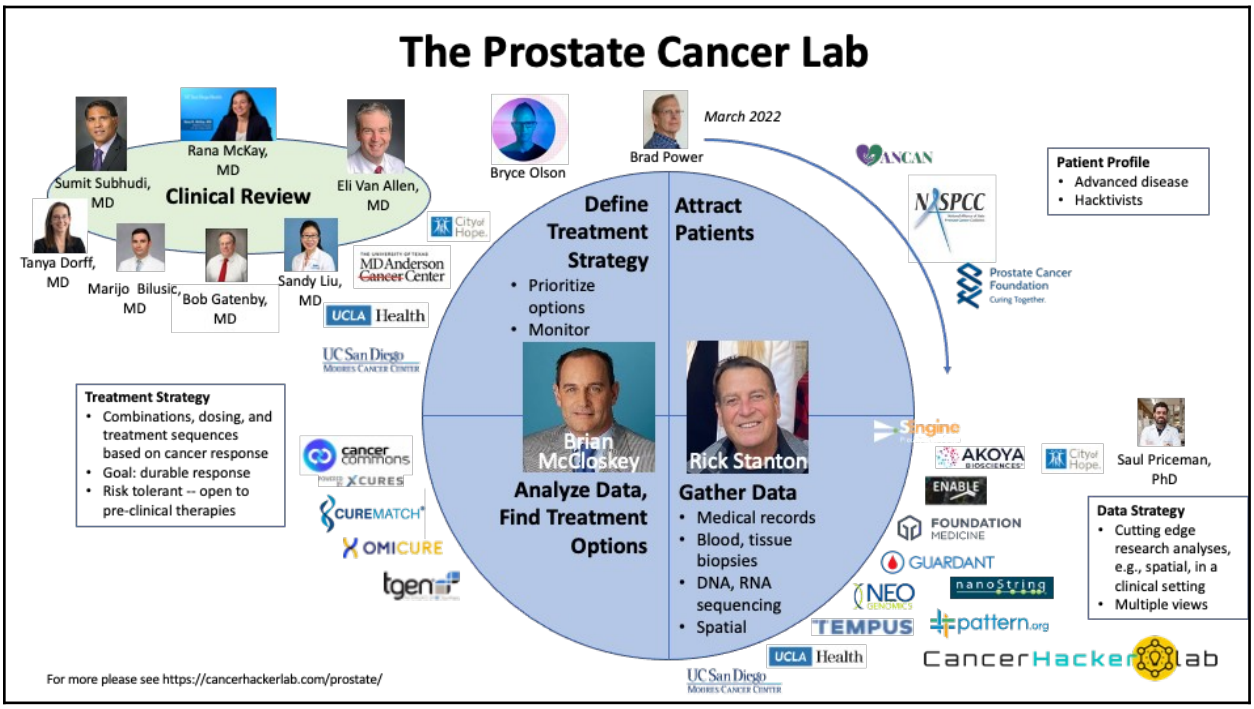
Rick Stanton: I’d like to add color to that. We had a guy at Jefferson with a doc that was pushing him into an unsuitable trial. At least we thought it was unsuitable. They insisted on doing a biopsy. This doc was resistant to sending a sample to FMI for NGS, but we kept pushing the patient to advocate for himself. He went in for the biopsy about four weeks ago, and we said, “make sure to tell them that they take enough tissue, not just for the trial, but you want to have enough to send to FMI.” So he did. Now he had already gotten a liquid biopsy, but as most of you are probably aware, you don’t get a good MSI reading from blood biopsies. He needed a tissue biopsy to see if he was MSI high or not. So they took it, and he had to push them and push them, and eventually after a week and a half they sent off what they had, and then he didn’t hear anything. Why didn’t he hear anything? It turned out there wasn’t sufficient tissue for FMI to do an analysis. It’s outrageous. He finished up switching docs. He went in for a bone biopsy last week. And now there’s tissue at FMI.

Brad Power: We’ve run four hackathons, and “the issue is tissue” has been a recurring theme, and tissue management. Whenever you go outside of these institutions, whenever you send it out, the pathologists have handoff issues. It’s been another recurring theme.

I’d like to share a poster of the prostate cancer lab process:

“Launch Meeting: Introductions and Purpose” (Brian McCloskey, Rick Stanton, Brad Power) [#1]

Exhibit 5: Prostate Cancer Lab process



This gives you a sense of the cycle we are launching for Brian and Rick.
You see quite a number of companies on the “gather data” step, consistent with what we’ve been talking about in trying to get as much data and information as possible to inform decisions. We are adding to this all the time.

On the “attract patients” step you see AnCan, and Brian can speak to our relationship with the Prostate Cancer Foundation and the National Alliance of State Prostate Cancer Coalitions to get more patients involved in this cycle with us.

On the “analyze data” step you see TGen, and our friends at CureMatch, OmiCure, Cancer Commons, and xCures.

And some of the doctors who said they will help us with clinical review of the treatment options and defining a treatment strategy.

Brian McCloskey: I am now on the board of the National Alliance for State Prostate Cancer Coalitions. I just spoke in January in DC about my journey and the decision-making process. They are a coalition of state prostate cancer groups. I’m getting to know them. They produce Prostatepedia, which is a phenomenal resource for prostate cancer patients. All articles are written by experts in their field. It is an amazing magazine.

I have worked with the Prostate Cancer Foundation on a number of fronts. With Laura Kleiman, [founder and CEO of RebootRx, which looks at new uses for existing drugs], who was on here,

“Launch Meeting: Introductions and Purpose” (Brian McCloskey, Rick Stanton, Brad Power) [#1]

we were working with Howard Soule [former CEO of PCF] and others at PCF on an AI-enabled repurposing of drugs for prostate cancer.

More recently I have gotten involved with Gina Carithers, the founder of Urotoday [and involved in communications at PCF]. She introduced me to Chuck Ryan, the CEO of the Prostate Cancer Foundation. Chuck was super helpful in terms of providing guidance on how to look at diagnostics. He was very receptive. I spent 90 minutes with him on the phone about a couple weeks ago. We will reconvene in about a month. He needs to process some things, and I need to follow up on some leads he gave me.

Rick Stanton: I would like to give anyone on the call a chance to introduce themselves, say hello, and thank them.

Lea Ann Biafora: It’s an honor to be with you all today. As an oncology nurse of thirty years, I started a company [Beacon Associates] ten years ago helping cancer patients navigate the healthcare system. With all of the genomics, it has become very complex in all cancers. I work with some of the scientists and physicians on today’s call. I’m grateful to be a part of this and to have this opportunity to learn. I have a couple of patients I’m thinking about to refer to you. We experience the issue of tissue nationally, and just the issue of testing – getting physicians to test. We want to get more access to the technologies you’re talking about to more people.

Jeff Waldron: I think I know a lot of the people on this call. I’m not a clinical person, but I can make connections. I have been involved in expanded access programs for explorational therapies.

Stacy Hurt: I’m in Pittsburgh, PA. I was treated through UPMC. I’m happy to help if you need anything here in Pittsburgh. I’m a Stage IV colon cancer survivor. I had a really rough battle. I had cancer across 27 places in my body: liver, rectum, lymph nodes, a less than 10% chance of survival. I was very lucky: I was an exceptional responder to standard treatment, FOLFOX [chemo]. I have a child with a rare disease, so my advocacy stretches across rare disease as well as oncology. I have a passion for our fellow patients and am blessed to be well connected, so here to help, learn, listen, and support in any way I can.

Jason Sager: I’m a pediatric oncologist by training. I have seen how well pediatric oncology did in organizing and getting patients on successive rounds of information gathering and treatment options that left a great improvement over the lifetime that I have been working in the clinic. Looking at adult oncology, they have a lot to gain from that. Efforts like this are a vision of the future of what we hope every patient can access. My hat is off to you guys. I hope I can help in some way, shape, or form.

Jan Sobieralski: I’m 72-years-old. I live on Mercer Island near Seattle. I was diagnosed in 2018. Gleason 9. Prostate cancer metastasized to the bones. I had proton radiation in 2019 that seemed to eradicate my cancer that was in my anal region. Now it is just in the bones. My PSA seems to be under control. I was on firmagon for a while, but went off it due to side effects. I

went on an estrogen topical cream. I was on Lupron and started having heart issues with it. It has kept my PSA at less than .01 for a couple years. I let it rise when I had surgery for my colorectal cancer. Now it is coming back down. I am not in that ADT-resistant category yet, but I am anticipating that could be happening.

Brad Power: We will be on a weekly cycle of meetings and notes.

Brain McCloskey: Thanks everyone for joining.